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File No.: EXP202313165
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RESOLUTION OF RIGHTS PROCEDURE

Having reviewed the complaint registered on September 5, 2023, with this Agency and having carried out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: A.A.A. (hereinafter, the complainant) exercised the right of Access to their medical history against the HEALTH SERVICE OF CASTILLA-LA MANCHA (SESCAM) (hereinafter, the respondent) without their request receiving the legally established response.

The complainant alleges that they have not received a response to their request for a copy of their medical history. They request access to it, to the traceability of who has accessed said history, or, failing that, the physical location from which it has been accessed.

SECOND: In accordance with Article 65.4 of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), this complaint was forwarded to the respondent for analysis and to inform this Agency within one month of the actions taken to comply with the requirements established in data protection regulations.

THIRD: The outcome of the transfer procedure indicated in the previous Fact did not allow for the understanding that the complainant's claims were satisfied. Consequently, on December 5, 2023, for the purposes provided for in Article 64.1 of the LOPDGDD, the Director of the Spanish Agency for the Protection of Data agreed to admit the complaint for processing.

The aforementioned agreement granted the respondent a hearing period, to submit any allegations they deemed appropriate within ten working days.

The respondent states:

"...In relation to file number (...), proof of sending from the Guadalajara Management Directorate to the complainant of a copy of their medical history is attached, in response to their request for the exercise of rights. Furthermore, it is indicated from the aforementioned Directorate that regarding the request for traceability of who has accessed the complainant's medical history, or, failing that, the physical location from which it has been accessed, the corresponding report is being prepared..."

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FOURTH: After examining the document submitted by the respondent, it is forwarded to the complainant, so that, within ten working days, they may submit any allegations they consider appropriate. The complainant has not submitted any allegations.

LEGAL GROUNDS

I

Competence

In accordance with the powers granted to each supervisory authority by Article 58.2 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), and as established in Articles 47, 48.1, and 64.1 of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the Director of the Spanish Agency for the Protection of Data is competent to initiate and resolve this procedure.

Furthermore, Article 63.2 of the LOPDGDD states: "The procedures processed by the Spanish Agency for the Protection of Data shall be governed by the provisions of Regulation (EU) 2016/679, by this organic law, by the regulatory provisions issued in its development, and, in so far as they do not

contradict them, subsidiarily by the general rules on administrative procedures."

II

Preliminary Issues

In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for the Protection of Data is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among controllers and processors regarding their obligations, as well as addressing complaints filed by a data subject and investigating, as appropriate, the reasons for them.

Correspondingly, Article 31 of the GDPR establishes the obligation of controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In cases where they have appointed a data protection officer, Article 39 of the GDPR assigns this officer the function of cooperating with that authority.

In accordance with this regulation, following the procedure provided for in Article 65.4 of the LOPDGDD, prior to the admission for processing of the complaint regarding the exercise of the rights regulated in Articles 15 to 22 of the GDPR, which gives rise to this procedure, it was forwarded to the respondent for analysis, to respond to this Agency within one month, and to demonstrate that they had provided the complainant with the due response.

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3/8

The result of this transfer did not allow for the satisfaction of the claims of the complaining party.

Consequently, on December 5, 2023, for the purposes provided in Articles 64.1 and 65 of the LOPDGDD, the complaint submitted was admitted for processing. This admission for processing determines the opening of the present procedure regarding the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, according to which:

"When the procedure refers exclusively to the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679 of the European Parliament and of the Council, of April 27, 2016, it shall be initiated by an admission agreement, which shall be adopted in accordance with the provisions of Article 65 of this organic law.

In this case, the deadline for resolving the procedure shall be six months from the date on which the agreement of admission for processing was notified to the complainant. After this period, the interested party may consider their complaint accepted."

Article 58.2 of the GDPR grants the Spanish Data Protection Agency a series of corrective powers for the purpose of rectifying any infringement of the GDPR, including "ordering the controller or processor to address the requests for the exercise of the rights of the data subject under this Regulation."

III

Rights of Individuals Regarding Personal Data Protection

The rights of individuals regarding personal data protection are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. The rights of access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability are contemplated.

The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Additionally, consideration is given to what is expressed in Recitals 59 and following of the GDPR.

In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of rights by the data subject, which shall be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless it can demonstrate that it is unable to identify the data subject, and to express its reasons in case it does not intend to address such request. The burden of proof regarding compliance with the obligation to respond to the request for the exercise of

rights made by the affected party lies with the controller.

The communication addressed to the data subject regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language.

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4/8

IV

Right of Access

In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the data subject has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data."

Regarding the right of access to personal data, according to the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the data controller may request the data subject to specify the "data or processing activities to which the request refers." The right will be considered granted if the controller provides remote access to the data, with the request being deemed fulfilled (although the data subject may request information regarding the aspects provided for in Article 15 of the GDPR).

The exercise of this right may be considered repetitive if it is exercised more than once within a period of six months, unless there is a legitimate reason for it. On the other hand, the request will be considered excessive when the data subject chooses a means other than the one offered that incurs a disproportionate cost, which must be borne by the data subject.

Like the other rights of the data subject, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them that is being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the data may be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the information available about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed will not negatively affect the rights and freedoms of others.

The right of access concerning medical records is specifically regulated in Article 18 of Law 41/2002, of November 14, which is the basic regulation of Patient Autonomy and Rights and Obligations in the Field of Information and Clinical Documentation (hereinafter LAP), the literal text of which states:

"1. The patient has the right of access, with the reservations indicated in section 3 of this article, to the documentation of the medical record and to obtain a copy of the data contained therein. Health centers will regulate the procedure that guarantees the observance of these rights.

2. The patient's right of access to the medical record may also be exercised by duly accredited representation.

3. The patient's right of access to the documentation of the medical record may not be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the right of the professionals involved in its preparation, who may oppose the right of access based on the confidentiality of their subjective notes.

4. Health centers and individual practitioners will only facilitate access to the medical records of deceased patients to persons linked to them, for family reasons or, in fact, unless the deceased expressly prohibited it and this is accredited. In any case, access by a third party to the medical record motivated by a risk to their health will be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals, or that harms third parties, will not be provided."

In this regard, it is important to highlight Article 15 of the LPA, which outlines the minimum content of the medical record:

"1. The medical record will incorporate the information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in the primary care and specialized care fields.

2. The main purpose of the medical record will be to facilitate healthcare, documenting all those data that, under medical criteria, allow for the truthful and updated knowledge of the health status.

The minimum content of the medical record will be as follows:

a) Documentation related to the clinical-statistical sheet.

b) Admission authorization.

c) Emergency report.

d) Anamnesis and physical examination.

e) Progress notes.

f) Medical orders.

g) Consultation sheet.

h) Reports of complementary examinations.

i) Informed consent.

j) Anesthesia report.

k) Operating room report or delivery record.

l) Pathological anatomy report.

m) Nursing care progress and planning.

n) Therapeutic nursing application.

ñ) Vital signs chart.

o) Clinical discharge report.

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6/8

Paragraphs b), c), i), j), k), l), ñ), and o) will only be enforceable in the completion of the medical history when it concerns hospitalization processes or as otherwise provided.

3. The completion of the medical history, in aspects related to direct patient care, will be the responsibility of the professionals involved in it.

4. The medical history will be maintained with criteria of unity and integration, at each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the healthcare providers of the data of a particular patient in each care process.

Regarding the preservation of the medical history, Article 17 of the LPA, in its points 1 and 5, states that:

"1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original format, for the adequate care of the patient for the appropriate time in each case and, at a minimum, five years from the date of discharge from each care process (...)

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate."

V

Conclusion

Even before the admission for processing that led to this resolution, the respondent stated:

"...the interested party has submitted different requests, all with the same objective, as the interested party herself acknowledges in one of them, 'to know which individuals have accessed her medical history,' and not as indicated in the complaint, a request for a copy of her medical history.

The first of the requests was submitted by the claimant on February 18, 2022. As has been verified, this request was resolved by the competent body through a Resolution dated February 18, 2022, which dismissed her request regarding the delivery of access, as it required data that is not related to

her person, but affects third parties (to know the identity of the professionals who have accessed her medical history), which is not covered by the exercise of the right of access, indicating, however, that the aforementioned Resolution will check if there have been accesses that can be classified as improper, acting through the appropriate disciplinary and/or jurisdictional procedures, if applicable. In compliance with the provisions of the aforementioned Resolution dated February 18, 2022, from the Management Directorate of the Integrated Area of Guadalajara, it proceeds to review the accesses to the medical history of the claimant, in order to determine, if applicable, possible improper accesses, issuing a report dated

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7/8

December 23, 2022, in which it is determined that no unauthorized access has occurred in the medical record of the claimant.

Therefore, as a result of this response, the reason for admitting this claim for processing was the lack of accreditation by the respondent regarding the content sent to the claimant, considering that the documentation provided by the claimant proves that the right of access to the medical record was indeed exercised by them and that this right of access was not responded to.

During the processing of this procedure, the respondent entity has replied to this Agency stating that it has attended to the right and substantiating it with a proof of sending to the claimant, but not with a copy of what was sent. Furthermore, it claims that it will verify the alleged inappropriate access to the medical record by third parties.

Given, on the other hand, that the respondent party has not provided a copy of the necessary communication that must be directed to the claimant informing them about the decision made regarding the request to exercise the right of access to the medical record, and the corresponding proof of delivery, this claim is deemed valid.

However, it should be specified that this ruling does not extend to the information about the individuals who have accessed the medical record of the claimant, also requested by them, as this is information not included in the right of access to the medical record, in accordance with the provisions of Article 19.2 of Decree 24/2011, of April 12, 2011, on health documentation in Castilla-La Mancha, according to which:

"The patient's right of access to the data of their medical record does not include information about the personal data of individuals who, within the organization of the data controller, may have accessed it in the exercise of their functions."

In view of the cited provisions and others of general application, the Director of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO ESTIMATE the claim made by A.A.A. considering that the provisions of Article 15 of the GDPR have been infringed and to urge the HEALTH SERVICE OF CASTILLA-LA MANCHA (SESCAM) with NIF Q4500146H, to send to the claimant, within ten working days following the notification of this resolution, a certification addressing the requested right or a reasoned denial indicating the reasons why the request cannot be granted, in accordance with the provisions established in the body of this resolution. The actions taken as a result of this Resolution must be communicated to this Agency within the same period. Non-compliance with this resolution could result in the commission of an infringement of Article 83.6 of the GDPR, classified as very serious for the purposes of prescription in Article 72.1.m) of the LOPDGDD, which will be sanctioned in accordance with Article 58.2 of the GDPR.

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SECOND: NOTIFY this resolution to A.A.A. and to the HEALTH SERVICE OF CASTILLA-LA MANCHA (SESCAM).

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file an administrative contentious appeal before the Administrative Court of the National Court, in accordance with the provisions of Article 25 and Section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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